Schedule of Activities: Screening and Treatment

	Pre-Screening	[2] Screening and Post Randomization		Lymphodepletion			Treatment and Follow-up													
[1] Visit Identifier		[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]
Visit Window				[redacted]					[redacted]											
[3] Informed consent		X																		
Tumor history		X																		
[4] Medical history		X																		
[5] Viral/bacterial/protozoal assessment		X																		
Eligibility criteria		X																		
Clinical Evaluation																				
[6] Physical examination		X		X	X	X	X			X		X	X	X	X	X	X	X	X	X
[7] Vital signs (BP/pulse rate/Temp/O_2 Sat)		X		X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
ECOG performance status		X					X							X		X	X	X	X	X
[8] ICE assessment/ ICANS		X					As clinically indicated													
[9] 12 lead ECG		X					X							X						X
ECHO or MUGA		X		As clinically indicated																
Local Laboratory																				
[10] ALLO-501A Donor-specific anti-HLA antibodies: screening (central lab)	(X)	(X)																		
[11] CBC w/differential		X		X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
[12] Blood Chemistry		X		X			X			X		X		X		X	X	X		X
[13] Coagulation		X					As clinically indicated (e.g. CRS)													X
[14] Reflex autoantibodies				In case of suspected ALLO-647 treatment emergent autoimmunity																
Blood culture (aerobic and anaerobic)			X			X														
[15] Urinalysis		X												X		X	X	X	X	X

CONFIDENTIAL

	Pre-Screening	[2] Screening and Post Randomization		Lymphodepletion			Treatment and Follow-up													
[1] **Visit Identifier**		[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	**9**
Visit Window				[redacted]					[redacted]									[redacted]	[redacted]	[redacted]
[16] Pregnancy test and contraception check		X																	X	X
[17] RCL			X															X	X	X
TSH		X																X		X
CMV Antigen by PCR							X			X		X	X	[redacted]	[redacted]	X				
EBV and adenovirus monitoring				Per institutional practices																
[18] Quantitative serum immunoglobulins (local)			X				X					X		X		X	X	X	X	X
[19] Lymphodepletion																				
[19] Fludarabine (FC and FCA)				X	X	X														
[19] Cyclophosphamide (FC and FCA)				X	X	X														
[19] ALLO-647: FCA-90 (FCA only)				X	X	X														
Treatment																				
ALLO-501A							X													
Anti-infection prophylaxis				Based on Section 5.12.3; NCCN guidelines																
Tumor Assessments																				
[20] PET/CT		X												X		X	X		X	X
[21] Brain MRI		X		As clinically indicated																
[22] Bone marrow aspirate and biopsy		X		If positive at [redacted], or as clinically indicated																
[23] Lymphoma biopsy		X								As clinically indicated										
[24] MRD assessment		X												X		X	X		X	X
Other Clinical assessments																				
Serious and non-serious adverse events		X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Concomitant treatments		X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X

CONFIDENTIAL

	Pre-Screening	[2] Screening and Post Randomization		Lymphodepletion			Treatment and Follow-up													
[1] Visit Identifier		Day -34	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]
Visit Window				[redacted]					[redacted]											
[25] EQ-5D (PRO)		X												X		X	X		X	X
[25] QLQ-C30 (PRO)		X												X		X	X		X	X
Cellular kinetics/PK assessments – central laboratory testing																				
[26] ALLO-501A in blood (flow cytometry)				X						X	X	X	X	X	X	X	X	X		
[27] ALLO-501A in blood (transgene)							X EOI only	X	X	X	X	X	X	X	X	X	X	X	X	X
[28] ALLO-501A in BM aspirate							As clinically indicated													
[28] ALLO-501A in lymph node biopsy									X	As clinically indicated										
[28] ALLO-501A and viral in CSF							As clinically indicated													
[29] ALLO-647 serum concentration				X	X	X	X		X		X									
[30] Fludarabine and Cyclophosphamide serum concentration				X	X	X	X	X												
[31] Cytokines and soluble proteins (serum)			X			X	X	X	X	X	X	X		X	X	X	X			
Immunogenicity assessments																				
[32] Anti-ALLO-501A ADA				X										X		X	X	X		X
[33] Anti-ALLO-647 ADA				X										X		X	X	X		X
[34] ALLO-501A donor-specific anti-HLA antibodies(serum)														X			X	X		X
Pharmacodynamic assessments																				
[35] TBNK (blood)			X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
[36] Gene expression profile (blood RNA)			X							X		X		X		X				X
[37] TCR sequencing (blood DNA)			X							X		X		X		X				X

CONFIDENTIAL

	Pre-Screening	[2] Screening and Post Randomization		Lymphodepletion			Treatment and Follow-up													
[1] Visit Identifier		[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]	[redacted]
Visit Window				[redacted]					[redacted]											
Other assessments																				
[38] Genomic safety biospecimen			X																X	
[39] Banked serum and plasma			X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
[40] PBMCs			X								X		X		X		X		X	X
[41] Focal CNA or Risk Score		X																		

Abbreviations: + = positive; → = ongoing/continuous event; ADA = anti-drug antibody; BM = bone marrow; BP = blood pressure; CBC = complete blood count; CMV = cytomegalovirus; CSF = cerebral spinal fluid; CT = computed tomography; d = days; DNA = deoxyribonucleic acid; EBV = Epstein-Barr virus; ECG = electrocardiogram; ECHO = echocardiogram; ECOG = Eastern Cooperative Oncology Group; EOS = end of study; EQ 5D = EuroQol 5-Dimension questionnaire; FC = fludarabine, cyclophosphamide; FCA = fludarabine, cyclophosphamide, and ALLO-647; HLA = human leukocyte antigen; ICANS = Immune effector cell-associated neurotoxicity syndrome; ICE = Immune Effector Cell-Associated Encephalopathy; MRI = magnetic resonance imaging; mos = months; MRD = minimum residual disease; MUGA = multi-gated acquisition scan; PBMC = peripheral blood mononuclear cells; PET = positron emission tomography; qPCR = quantitative polymerase chain reaction; PRO = patient reported outcomes; RCL= replication competent lentivirus; RNA = ribonucleic acid; TBNK = T, B, NK cells; TCR = T cell receptor; TSH = thyroid-stimulating hormone; wks = weeks.

1 [redacted].

2 **Screening and Post Enrollment (Randomization):** Screening procedures are to be obtained within[redacted]. Specific procedures to be completed[redacted] are listed under[redacted] Visit.

3 **Informed Consent:** Pre-screening or Screening Consent for this study must be obtained prior to undergoing any study specific procedures.

4 **Medical History:** Includes history of disease process other than the cancer under study (active or resolved) and concurrent illness. Includes any current medical treatments for any condition. Demographic data collection will take place as part of the Screening Visit and will include the collection of sex, age, race and ethnicity.

5 **Viral/bacterial/protozoal assessment** by IgG and IgM serology tests, and reflex PCR in case of IgM seropositive result whenever clinically appropriate, except for CMV for which IgG serology test and DNA PCR (instead of IgM serology test) should be performed: Cytomegalovirus (CMV), Epstein Barr virus (EBV), human immunodeficiency virus (HIV)-1, HIV-2, hepatitis B (HBV, HbsAG, antiHBc, antiHbs), hepatitis C (HCV). IGRA for Latent TB. For subjects living in or travelling to endemic areas screening should be considered for West Nile Virus, Strongyloides, Cocciomycosis, and, Histoplamosis. Serum Aspergillus galactomannan antigen should be sent in high-risk patients.

6 **Physical examination (PE):** Physical examination (with neurologic exam) will include weight and height. Height will only be required[redacted].

CONFIDENTIAL

7 **Vital Signs:** Includes temperature (oral, tympanic, temporal or axillary), blood pressure (BP), pulse rate and oxygen saturation level. Oxygen saturation will be evaluated by pulse oximetry. On [redacted], to be performed [redacted].

8 **ICE Assessment/ ICANS**. Conduct immune effector cell-associated encephalopathy (ICE) score at screening. Conduct ICE score and immune effector cell-associated neurotoxicity syndrome (ICANS) grading as clinically indicated, starting on [redacted] ICE and ICANS grading scales are provided in Appendix 8.

9 **12 Lead electrocardiogram (ECG):** At each time point, 12 lead ECG will be performed to determine cardiac safety and QTcF interval. On [redacted], ECG will be performed prior to investigational product administration. If the QTcF is prolonged (>500 msec), the ECG should be re-evaluated by a qualified person at the institution for confirmation.

10 **Donor-specific anti-human leukocyte antigen (HLA) antibodies (DSA): Screening:** Samples will be collected and sent to a central laboratory for processing and analysis after the subject signs the pre-screening or screening informed consent form. Donor-specific anti-HLA antibodies must be repeated prior to [redacted] if subject receives blood transfusion (ie, red blood cells, platelets, plasma) after an initial negative result to confirm eligibility.

11 **CBC Hematology:** complete blood count (CBC) with differential (should include % and absolute number of neutrophils, lymphocytes, monocytes, eosinophils, basophils, and reticulocyte count), hemoglobin, hematocrit, platelet counts. On[redacted], hematology will be performed [redacted]. On [redacted], samples will be collected [redacted] infusion.

12 **Blood Chemistry (fasting not required):** should include: alanine aminotransferase (ALT), albumin, alkaline phosphatase, aspartate aminotransferase (AST), bicarbonate, total bilirubin, blood urea nitrogen (BUN), chloride, serum calcium, creatinine, creatinine clearance, glucose, lactate dehydrogenase, magnesium, sodium, phosphorus or phosphate, total protein, potassium, uric acid, serum ferritin, C-reactive protein (CRP). On [redacted], chemistry will be performed prior to start of lymphodepletion treatment. On [redacted] samples will be collected prior to ALLO-501A infusion.

13 **Coagulation:** fibrinogen and D-dimer, partial thromboplastin time (PTT) and International Normalized Ratio (INR) or prothrombin time.

14 **Reflex auto-antibodies** will be evaluated in case of suspected ALLO-647 treatment emergent autoimmunity AE. Depending on the type of AE, the tests should include (but not limited to) serum antiphospholipid antibodies (APA), platelet-bound APA, anti-Factor VIII antibodies, anti-Thyroid Peroxidase (anti-TPO), anti-Thyroglobulin (anti-TG) and anti-thyroid stimulating hormone (anti-TSH receptor), Coombs test, antinuclear antibodies (ANA), perinuclear anti-neutrophil cytoplasmic antibodies (P-ANCA), anti-neutrophil cytoplasmic antibodies (cANCA), and/or anti-Sjogren's syndrome type B (anti-SSB).

15 **Urinalysis:** Urine dipstick for protein, glucose, and blood: If positive collect 24 hr and/or microscopic cell count (Reflex Testing).

16 **Pregnancy test and contraception check:** Serum or urine pregnancy test for females of childbearing potential. Follicle stimulating hormone test may be performed for post-menopausal women. Contraception use will be checked to confirm that contraception is used consistently and correctly.

17 **Replication competent lentivirus (RCL):** Whole blood samples will be collected to monitor for the presence of RCL-specific DNA sequences by a validated PCR-based assay. Samples may be analyzed for RCL only [redacted]; if negative, then remaining samples [redacted] may be collected and archived. Assess for RCL upon diagnosis of a new (secondary) malignancy.

18 **Quantitative serum immunoglobulins (local analysis):** Humoral immunity will be monitored by the measurement of serum IgG, IgA, and IgM levels. On [redacted], samples will be collected prior to investigational product administration.

19 **Lymphodepletion:**

- **Fludarabine (FC and FCA arms):** 30 mg/m^2/day fludarabine will be administered IV over approximately 15 to 30 minutes for 3 days, from Day -5 to Day -3. The fludarabine dose should be adjusted based on renal function; for subjects with creatinine clearance <80 mL/min, the fludarabine dose should be 20 mg/m^2 on each of the 3 dosing days or as per institutional guidelines.
- **Cyclophosphamide (FC and FCA arms):** 300 mg/m^2/day cyclophosphamide will be administered by IV over approximately 1 hr for 3 days, from Day -5 to Day -3.
- **ALLO-647 (FCA arm only):** Administer 30 mg/d IV over approximately 4 hrs each day. For FCA-90, 30 mg/day x 3 days, from Day -5 to Day -3.

20 **Lymphoma assessments by positron emission tomography (PET)/computerized tomography (CT):** tumor assessments will include all known or suspected disease sites. Disease-specific imaging as clinically appropriate will be performed as described in the study imaging manual. All scans will also be sent to a central radiology vendor, as specified in the study imaging manual. If a PET/CT was performed within [redacted] prior to Screening, then the results may be used for Screening disease assessment and a Screening PET/CT does not need to be done, providing there was no intervening therapy between the time of the PET/CT and Screening. Radiographic disease assessment will not be required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

21 **Brain MRI:** Subjects with a history of CNS disease: screening brain MRI must be stable as compared to previous brain MRI scan.

22 **Bone Marrow Biopsy and Aspirate:** Bone marrow biopsy and aspirate will be completed [redacted], and at the time complete response (CR) is suspected only if bone marrow infiltration is positive or unknown at screening. Samples (screening; others as indicated) will be banked, and may be analyzed for insertion site analysis, molecular analysis, and other similar analyses. Proceduralist should aim to achieve a 2cm core to best assess cellularity.

23 **Lymphoma biopsy:** An archival formalin-fixed paraffin-embedded (FFPE) cancer tissue block from initial diagnosis, or previously obtained cancer tissue biopsy obtained prior to screening, may be submitted in place of a fresh sample, if obtained within [redacted] and if the subject received no intervening systemic anti-cancer treatment during this period. Cancer tissue from cytological sampling (eg, fine needle aspirates, including FFPE cell pelletthe material) is not adequate and should not be submitted. The FFPE tissue blocks must be of sufficient size to obtain approximately 22-26 unstained slides which should be submitted to the central laboratory. For subjects who undergo a fresh biopsy [redacted], or an on-treatment biopsy, a total of 3 core biopsies are to be obtained when possible for translational analyses.

24 **MRD assessment**: A plasma sample will be collected for MRD analysis using ClonoSeq or equivalent testing.

25 **EQ-5D-5L (PRO) and QLQ-C30 (PRO):** Completion of these instruments is not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

26 **ALLO-501A in blood (flow cytometry):** A blood sample will be collected to quantify ALLO-501A by a validated flow cytometry-based cellular-kinetics assay on [redacted], and at the indicated time points by central laboratory. Phenotypic subsets of CAR + T cells such as memory, activated and exhausted populations may also be characterized.

27 **ALLO-501A in blood (transgene levels):** A blood sample will be collected into a tube optimized for DNA isolation on [redacted], at the end of infusion on [redacted] (ie, within 30 min after infusion), and at the indicated time points to quantify ALLO-501A transgene levels by a validated qPCR-based transgene assay, performed by central laboratory.

CONFIDENTIAL

28 **ALLO-501A detection in lymph node, BM, and CSF:** A sample of the indicated tissues will be evaluated to investigate the quantity of ALLO-501A in tissue compartments that may harbor tumor cells as clinically indicated. ALLO-501A may be measured by flow cytometry and/or qPCR-based assays by central laboratory. If subject experiences prolonged (>42 days) cytopenia a bone marrow aspirate will be collected for TCR clonality assessment, central CAR flow cytometry analysis, VCN and potential RNASeq. CSF will be collected, as clinically indicated, at any time if Grade 3 and above ALLO-501A related neurological AEs and/or suspected encephalitis are observed. In addition to the ALLO-501A detection, infectious workup (including HHV) may also be performed with the CSF sample collected.

29 **ALLO-647 pharmacokinetics (PK) assessment (FCA arm only):** Blood samples will be collected at each time point for PK analysis of ALLO-647. ALLO-647 serum concentration will be measured by immunoassay during and following ALLO-647 infusions at the following time-points: FCA-90: on [redacted] pre-dose, and end of infusion (within 15 minutes), [redacted] pre-dose, and end of infusion (within 15 minutes), [redacted] pre-dose, and end of infusion (within 15 minutes). [redacted] collect sample pre-ALLO-501A infusion, and at subsequent days as shown in the schedule. Testing will be performed at a central laboratory.

30 **Fludarabine and cyclophosphamide pharmacokinetics (PK) assessment (FC and FCA arms):** Blood samples will be collected at each time point for PK analysis of fludarabine and cyclophosphamide. Fludarabine and cyclophosphamide concentration will be measured by immunoassay during and following fludarabine and cyclophosphamide infusions at the following time-points: on [redacted] pre-dose, end of fludarabine infusion (within 15 minutes), end of cyclophosphamide infusion (within 15 minutes), [redacted] pre-dose, end of fludarabine infusion (within 15 minutes), end of cyclophosphamide infusion (within 15 minutes), [redacted] pre-dose, and end of fludarabine infusion (within 15 minutes), end of cyclophosphamide infusion (within 15 minutes). [redacted] collect sample pre-ALLO-501A infusion, and at subsequent days as shown in the schedule. Testing will be performed at a central laboratory.

31 **Cytokines and Soluble Proteins:** Analysis will be completed at a central lab. Serum cytokines assessed for cytokine release syndrome (CRS) safety management will be managed locally. On [redacted], the cytokine assessment (blood) will be performed prior to start of lymphodepletion treatment. On [redacted], samples will be collected prior to ALLO-501A infusion.

32 **Anti-ALLO-501A Antibodies (anti-scFv and anti-TALEN antibodies):** A blood sample will be collected into a tube optimized for serum isolation to monitor for the development of ALLO-501A and TALEN-specific antibodies. Testing will be performed at a central laboratory.

33 **Anti-ALLO-647 Anti-Drug-Antibodies (FCA arm only):** circulating levels of human anti-ALLO-647 antibodies will be assessed in serum by immunoassays up to [redacted] or up to 2 consecutive negative tests post-dosing completion whichever is the latest. Neutralizing antibody testing will be performed only in case of positive anti-drug antibody (ADA). Testing will be performed at a central laboratory.

34 **Donor-specific anti-human leukocyte antigen (HLA) antibodies (DSA): (On study):** Binding of C1q to HLA Class I and II DSA will be tested in case of positive DSA results following [redacted].

35 **TBNK Assay:** A blood sample will be collected for the enumeration of T, B, and natural killer (NK) cells by a flow cytometry-based assay to monitor the depth and duration of lymphodepletion. Samples will be collected before administration of ALLO-647 ([redacted]) or ALLO-501A ([redacted]). Testing will be performed at a central laboratory.

36 **Gene expression (RNA) profile:** Gene expression in whole blood associated with CART cell phenotype, activation state, and anti-tumor activity will be measured by RNA sequencing in blood, and bone marrow if bone marrow is collected to monitor response. Testing will be performed at a central laboratory.

37 **TCR sequencing:** Whole blood samples will be collected into a tube optimized for isolation of DNA at the indicated time point to assess the clonality of ALLO-501A, the subject T cell repertoire by TCR sequencing analysis by a central laboratory. Testing will be performed at a central laboratory.

38 **Genomic safety biospecimen:** Unless prohibited by local regulations or ethics committee decision, a blood sample will be collected during [redacted], and [redacted], for assays related to treatment response in the disease under study, e.g. insertion site analysis, malignancy panels, microbial cell–free DNA (mcfDNA) for pathogens that cause infections, etc.

39 **Banked serum and plasma samples:** Serum and plasma samples will be collected at the indicated timepoints and banked for future studies including endemic infectious diseases (e.g. HHV-6/7).

40 **PBMCs:** Peripheral blood mononuclear cells (PBMCs) will be collected at the indicated timepoints and cryopreserved for future cytogenetics, cellular, molecular or proteomic studies.

41 **Focal CNA or risk score**: A plasma sample will be collected for focal copy number alteration (CNA) or risk score (Cherng et al, 2022).

CONFIDENTIAL

SCHEDULE OF ACTIVITIES: Follow-up and End of Study

	Follow-up		EOS
Visit Identifier	[redacted]		
[1] **Visit Window**	[redacted]		
[2] Physical examination	X	X	X
ECOG performance status	X	X	X
[3] CBC w/differential	X	X	X
[4] Blood Chemistry	X	X	X
[5] TSH (FCA arm only)	[redacted]	X	X
[6] Urinalysis (FCA arm only)	[redacted]	X	X
[7] Pregnancy test and contraception check	[redacted]		
[8] Reflex autoantibodies	As clinically indicated		
[9] EQ-5D (PRO)	X	X	X
[9] QLQ-C30 (PRO)	X	X	X
Tumor assessments			
[10] PET/CT	X	X	X
Brain MRI	As clinically indicated		
[11] Bone marrow aspirate and biopsy	As clinically indicated		
[12] Lymph node biopsy	As clinically indicated		
Other assessments			
Serious and non-serious adverse event monitoring	See Section 8		
Concomitant treatment(s)	See Section 5.11		
[13] Follow-up for survival and subsequent anticancer treatment (telephone contact if visits are discontinued)	X	X	X
[14] RCL	[redacted]	[redacted]	X
[15] Genomic safety biospecimen	[redacted]	X	X
[16] PBMCs	[redacted]	X	X

1 [redacted].

2 **Physical examination (PE):** Physical examination will include weight. Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

3 **CBC Hematology:** complete blood count (CBC) with differential (should include % and absolute number of neutrophils, lymphocytes, monocytes, eosinophils, basophils, and reticulocyte count), hemoglobin, hematocrit, platelet counts. Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

CONFIDENTIAL

4 **Blood Chemistry (fasting not required):** should include: alanine aminotransferase (ALT), albumin, alkaline phosphatase, aspartate aminotransferase (AST), bicarbonate, total bilirubin, blood urea nitrogen (BUN), chloride, serum calcium, creatinine, creatinine clearance, glucose, lactate dehydrogenase, magnesium, sodium, phosphorus or phosphate, total protein, potassium, uric acid, serum ferritin, C- reactive protein (CRP). Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

5 **TSH (FCA arm only)**

6 **Urinalysis (FCA arm only):** Urine dipstick for protein, glucose, and blood: If positive collect 24 hr and/or microscopic cell count (Reflex Testing)

7 **Pregnancy test and contraception check:** Serum or urine pregnancy test for females of childbearing potential. Follicle stimulating hormone test may be performed for post-menopausal women. Contraception use will be checked to confirm that contraception is used consistently and correctly.

8 **Reflex auto-antibodies** will be evaluated in case of suspected ALLO-647 treatment emergent autoimmunity AE. Depending on the type of AE, the tests should include (but not limited to) serum antiphospholipid antibodies (APA), platelet-bound APA, anti-Factor VIII antibodies, anti-Thyroid Peroxidase (anti-TPO), anti-Thyroglobulin (anti-TG) and anti-thyroid stimulating hormone (anti-TSH receptor), Coombs test, antinuclear antibodies (ANA), perinuclear anti-neutrophil cytoplasmic antibodies (P-ANCA), anti-neutrophil cytoplasmic antibodies (cANCA), and/or anti-Sjogren's syndrome type B (anti-SSB).

9 **EQ-5D-5L (PRO) and QLQ-C30 (PRO):** Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

10 **PET/CT Lymphoma assessments by positron emission tomography (PET)/computerized tomography (CT):** tumor assessments will include all known or suspected disease sites. Disease-specific imaging as clinically appropriate will be performed as described in the study imaging manual. Radiographic disease assessment will not be required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

11 **Bone Marrow Biopsy and Aspirate:** Bone marrow biopsy and aspirate will be completed at the time complete response (CR) is suspected only if bone marrow infiltration is positive or unknown at screening, and as clinically indicated. Samples (screening; others as indicated) will be banked, and may be analyzed for insertion site analysis, molecular analysis, and other similar analyses. Proceduralist should aim to achieve a 2cm core to best assess cellularity. Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

12 **Lymph node biopsy:** To be performed as clinically indicated. Not required after documented disease progression per Lugano criteria or initiation of subsequent anti-cancer therapy.

13 **Follow-up for survival:** All randomized subjects will be followed in the follow-up period for survival and disease status. As part of the study sites may be asked to conduct searches of public records, such as those establishing survival status, if available, to obtain survival data if the subject survival status is not already known. Sites may be also asked to also retrieve autopsy reports to confirm status of disease at the time of death and as per FDA Guidance: Testing of Retroviral Vector-Based Human Gene Therapy Products for Replication Competent Retrovirus During Product Manufacture and Patient Follow-up (July 2018)

14 **Replication competent lentivirus (RCL):** Whole blood samples will be collected to monitor for the presence of RCL-specific DNA sequences by a validated PCR-based assay. Samples may be analyzed for RCL only [redacted]; if negative, then remaining samples [redacted] may be collected and archived. Assess for RCL upon diagnosis of a new (secondary) malignancy.

15 **Genomic safety biospecimen:** Unless prohibited by local regulations or ethics committee decision, a blood sample will be collected during [redacted] and every [redacted], for assays related to treatment response in the disease under study, eg, insertion site analysis, malignancy panels, etc.

16 **PBMCs:** Peripheral blood mononuclear cells (PBMCs) will be collected at the indicated timepoints and cryopreserved for future cytogenetics, cellular, molecular or proteomic studies.

CONFIDENTIAL